جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : داليا على السيد احمد
المحافظة : القاهره **الحي / المدينة :** مصر الجديده **العنوان :** السبع عمارات
رقم البطاقة : 26910010103406 **رقم التليفون :** 01284485533 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 01-10-1969 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام اشعة مقطعية مخ مع صبغة [10851] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: داليا على السيد احمد
National ID: 26910010103406

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Contrast-enhanced CT of the brain**.

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
داليا
على السيد احمد على
١ش الشهيدمصطفى طبالة-السبع عمارات
مصر الجديده - القاهره
٢٦٩١٠٠١٠١٠٣٤٠٦
JB4720762


٢٦٩١٠٠١٠١٠٣٤٠٦ ٢٠٢١/٠٩
حاصله على معهد فنى تجارى
انثى مسلمة متزوجة
محمد ابراهيم محمد الطباخ
١١ البطاقة سارية حتى ٢٠٢٨/٠٩/١٩

National ID - Front & Back
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جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

أسبوع المصرية هي صحة مصر

G

MDT FORM 11/2/2026

Name	دالیا علي السيد	MRN	1461/2026
		ID	٢٩١٠١٠١٢٤٠٦
Age	56	Phone	٠١٢٨٤٤٨٠٠٢٢
Menopausal status	post menopausal	Marital status	married
NRS SCORE	PS 0-1	Family history	+ve PH (mother, sister, father)
BMI		REASON OF MDT	New case
CO morbidity	- HTN , C-section , Pelvic tube removal		

Clinical Data

BX: P/c omorphic metastatic lobular carcinoma to peritoneum & ovaries)

ER:	PR:	HER 2:	Ki67:
8/8	8/8	+1	

Sonomammography: Intially : constipation + anemia upper GI pyloric obstruction from mass outside & biopsied , lower GI Prob

Mets Workup: 5/1/26 → Pathology from Rt. ovarian mass CCT guided : metastatic → single site ovarian biopsy
P/c omorphic lobular carcinoma
18/01 → PET-CT : - duodenal circumferential thickening
- active dense ascites + peritoneal thickening

R L

TNM
- retro peritoneal soft tissue mass lesion
- Rt. pelvic soft tissue mass
- increase in bone marrow FDG uptake

Recommendations: Pt took 1 cycle → Taxol carboplatin , next week C2 in 5 days

CA 125 → 1566 CA 19-9 : 158

Pt start Tax/Carb weekly X 2 wk at side — I do rem to contact for pt's JCC H.O

MDT OUTCOME & PLAN weekly Taxol Tx

Pt forward X3 mo for Re-evaluation

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric





٠١٢٨٢٤٨٥٥٣٣

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : داليا علي السيد احمد علي **Analysis ID :** 1023474

Age : 56 Y : 3 M : 6 D **Gender :** Female **Conf. Code :** 4344

Referred By Prof. Dr. : عمرو زايد

Clinical Diagnosis : Right ovarian mass lesion with multiple peritoneal mass lesions.

Nature of Specimen : Block (CT guided biopsy, labelled as 1022773)

Receiving Date : 2026-01-05 **Patient ID :** 752471

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block then treated by monoclonal antibodies and detection kit (EnVision FLEX) using DAB as chromogen and hematoxylin as counter-stain,using Omnis machine.
The sections were treated against ER (Estrogen receptor) \ PR
Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain
The sections were treated against: Ck 7 \ Ck 20 \ CDX2 \ PAX8 \ GATA 3 \ HER2neu.

Results :

Tumor cells are STRONGLY POSITIVE for Ck 7.
Tumor cells are NEGATIVE for Ck 20.
Tumor cells are NEGATIVE for CDX2.
Tumor cells are NEGATIVE for PAX8
Tumor cells are STRONGLY POSITIVE for GATA 3.
95% of tumor cells show STRONGLY POSITIVE Staining for Estrogen receptors
90% of tumor cells show STRONGLY POSITIVE Staining for progesterone receptors.
Sections treated for HER2 revealed MINIMAL INCOMPLETE FAINT CELL MEMBRANE STAINING IN MORE THAN 10% OF TUMOR CELLS.

Conclusion :

Block, FINDINGS ARE COMPATIBLE WITH METASTATIC PLEOMORPHIC LOBULAR CARCINOMA OF BREAST ORIGIN.
POSITIVE FOR ESTROGEN RECEPTORS (SCORE 8/8)
POSITIVE FOR PROGESTERONE RECEPTORS (SCORE 8/8)
HER 2 LOW, SCORE 1+.

Prof. Dr. Shady Elia Anis *Prof. Dr. Elia Anis Ishak*

DR ELIA ANIS LABS

This report is electronically signed , any abrasion or modification would render it void .

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

Pathology Report
[[DOCTYPE: PATHOLOGY_REPORT]]

Dr.
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Patient Name : داليا على السيد احمد على **Analysis ID :** 1022773

Age : 56 Y : 3 M : 1 D **Gender :** Female **Conf. Code :** 5438

Referred By Prof. Dr. : عمرو زايد

Clinical Diagnosis : Right ovarian mass lesion with multiple peritoneal mass lesion.

Nature of Specimen : CT guided biopsy

Receiving Date : 2026-01-03 **Patient ID :** 752471

PATHOLOGY REPORT

Gross :

Multiple fragmented greyish white tissue cores, the longest measured 2 cm, totally submitted.

Microscopic :

Sections examined from the specimen received revealed cores of smooth muscle tubal wall and fibrofatty tissue with infiltrating tumor tissue formed of groups of malignant epithelial cells showing moderate nuclear pleomorphism and hyperchromasia, arranged in signet ring. There is moderate desmoplasia and no necrosis.

Diagnosis :

Right ovarian mass lesion with multiple peritoneal mass lesion, CT guided biopsy, METASTATIC POORLY COHESIVE CARCINOMA WITH SIGNET RING FEATURES.
- CASE FOR IMMUNOPHENOTYPING.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

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DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Dr.
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Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : داليا على السيد احمد على **Analysis ID :** 1024046

Age : 56 Y : 3 M : 6 D **Gender :** Female **Conf. Code :** 7297

Referred By Prof. Dr. : رامى سعيد

Clinical Diagnosis : Circumferential hyperemic mucosa at junction between bulb and second part of duodenum.

Nature of Specimen : Endoscopic biopsy.

Receiving Date : 2026-01-08 **Patient ID :** 752471

PATHOLOGY REPORT

Gross :

Multiple greyish white tissue pieces, collectively measured 0.5x0.5 cm, totally submitted.

Microscopic :

Sections examined from the specimen received revealed pieces of duodenal mucosa with infiltrating tumor tissue formed of groups of malignant epithelial cells showing moderate nuclear pleomorphism and hyperchromasia, arranged in signet ring.
There is moderate desmoplasia and no necrosis.

Diagnosis :

Duodenal mass, Endoscopic biopsy, INFILTRATING SIGNET RING CARCINOMA.

Prof. Dr. Shady Elia Anis *Prof. Dr. Elia Anis Ishak*

This report is electronically signed , any abrasion or modification would render it void .

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

09/23/2026

MDT FORM

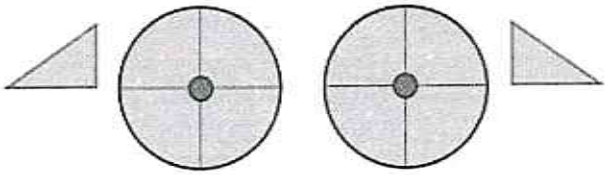
Name:	Dalia Aly Elsayed احمد	**MRN:**	7359
Age:	57 Y	**ID:**	26910010103406
Menopausal Sta	POST	**Phone:**	01225006475
NRS SCORE:		**Marital Status:**	+
BMI:		**Family History:**	+ VE BREAST CANCER
CO Morbidity:	HTN	**Reason Of MDT:**	CONTINUE TAXOL CARB

Clinical Data:

BX: LOBURAL CARCINOMA

ER: 8/8 **PR:** 8/8 **HER2:** 0 **KI67:**

R L



TNM

Sonomammography:
-VE AND P[ET-CT NEGATIVE

Mets Workup:

History:
A CASE PERTONEAL LOBULAR CARINOMA DIAGIONSED IN 1/2026 OF PERTONUM HR+ GER- RECEIVED 5 CYCLES TAXOL CARB WITN REGRESSION ON PET-CT SHIFT TO HT FEMRA IBRANCE DUE TO REPETED MYLOSUPRESSION RECIVED 2 (VOMITING AND INTOLARBILITY) SHIFTED AGAING TO TAXOL CARB /AVASTIN DR KHALED HUSSINY NO BASE LINE IMAGEING

received 3 cycles of taxol , carbo

PET CT 17/9/2026 : progressive course of low grade FDG avid annular mural thickening of 1st part of deudenum , mild regression of low grade FDG avid adenexal lesions

(severe allergic reaction of carbo occured to be omitted)
to continue 3 more cycles of taxol then evaluation ??

Recommendations:

MDT Outcome & Plan
For:
1.Bring pathology & IHC
2.If //progression of doudenal mural thickening is asymptomatic, maintain on Taxol.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د شيماء لاشين	د امنيه عيسوي				

23/9/2026 Omnia 23/9/2026

MDT Form
[[DOCTYPE:MDT_FORM]]

Visit Number:	7102652540	Client Name:	
Patient Name:	Dalia Ali El Sayed	Referred by Prof:	Khaled El Hossany
Age / Sex:	56 year / Female	Exam Date:	17 Sep 2025
Patient ID:	2140002823 Acc: 2160818	Printed:	18 Sep 2025 02:12:45 PM

PT

FOLLOW UP WHOLE BODY PET/CT EXAMINATION

History and Clinical Data:-

Patient data:-

- Female patient, 56 years old, Weight: 51 Kg, Blood Glucose level: 156 mg/dl.

Radiopharmaceutical:-

- F-18-FDG 4 mCi injected I.V.

Clinical background:-

- ? Metastatic breast & duodenal cancer.

Purpose of the study: -

- Assess response to therapy.

TECHNIQUE: -

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~1.6.
- Liver showed max. SUV~2.

PET Scan
[[DOCTYPE:PET_SCAN]]
Scanned with CamScanner

Visit Number:	71026525400	Client Name:	
Patient Name:	Dalia Ali El Sayed	Referred by Prof:	Khaled El Hosseny
Age / Sex:	56 year / Female	Exam Date:	17 Sep 2026
Patient ID:	2140002820 Acc 2150818	Printed:	18 Sep 2026 02:12:45 PM

PT

REPORT:-

Compared to the last study done on 2-4-2026.

Review of the PET, CT and the fused images revealed:-

I) Breasts:

- No gross FDG avid bilateral breasts masses. No FDG avid regional pathological axillary or internal mammary LNs.

II) Abdomino-pelvic cavity:

- **Progressive course regarding the size** of the low grade FDG avid circumferential mural thickening of 1st part of duodenum; reaching 2 cm in thickness with SUVmax 2.
- **No appreciable changes** regarding the previously seen low grade FDG avid retro-peritoneal & mesenteric ill-defined infiltrative soft tissue lesions seen encasing the abdominal aorta & proximal iliac arteries & attenuating the its branches; extending laterally to renal hila with consequent back pressure as well as into porta hepatis with resultant mild IHBRs dilation. It shows max. SUV~3.1.
- **Mild size regression** of the previously seen low grade FDG avid right adnexal ST lesions with max. SUV~2.8. It measures about 5.8x4.3 cm. Still seen bulky left adnexa.
- Diffuse smudging of the mesenteric & peritoneal fat with pre-sacral soft tissue sheets.
- No FDG avid pancreatic masses. The pancreatic duct is not dilated.
- No FDG avid gastric or colonic masses.
- Associated mild ascites (progressed).

III) The rest of the body:

- Average sized spleen showing normal FDG uptake with no metabolically active FDG avid focal lesions. New non-FDG avid hypodense geographical sub-capsular splenic lesions; suggestive of infarctions.
- Mild hepatomegaly showing normal FDG uptake with no metabolically active FDG avid focal lesions.
- Diffusely increased bone marrow FDG uptake; likely hyperplastic. No FDG avid lytic/sclerotic osseous lesions.
- Clear both lungs apart from bilateral basal atelectasis. No FDG avid pulmonary masses or nodules. No pleural effusion.
- No FDG avid renal or adrenal masses.

PET Scan
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Visit Number:	71026525400	Client Name:	
Patient Name:	Dalia Ali El Sayed	Referred by Prof:	Khaled El Hossany
Age / Sex:	56 year / Female	Exam Date:	17 Sep 2026
Patient ID:	2140002820 Acc 2160818	Printed:	18 Sep 2026 02:12:45 PM

PT

Other Findings:

- Stable few small left lower cervical/supra-clavicular LNs with no corresponding FDG activity.
- Diffuse sub-cutaneous edema.
- Right sided port-a-cath is seen with its tip at atrio-caval junction.
- Cardiomegaly.
- Enlarged thyroid showing diffusely increased FDG activity with max. SUV~6.2; likely due to thyroiditis. For clinico-laboratory correlation.
- Colonic diverticulosis.
- 1st degree spondylolisthesis of L4.
- Osteopenic bony texture.
- Uterine fibroid.

IV) The rest of PET / CT study appears normal.

V) Normal physiological tracer bio-distribution is seen over cerebral hemispheres, heart, bowel, bone marrow and urinary tract.

CONCLUSION:

- Follow up PET-CT study showing:

  No overt FDG avid bilateral breast masses or regional lymphadenopathy.

  Progressive course of the low-grade FDG avid annular mural thickening of 1st part of duodenum.

  No significant changes regarding the large low-grade FDG avid abdominal retro-peritoneal/mesenteric infiltrative ST lesions.

  Mild size regression of the low-grade FDG avid adnexal lesions.

  Progressed mild ascites.

Dr. Sherif Filfil. Prof. Ahmed Wafaie MD, FRCR

PET Scan
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Scanned with CamScanner



PET Scan

[[DOCTYPE:PET_SCAN]]

Scanned with CamScanner

Visit Number:	71026507395	Client Name:	
Patient Name:	Dalia Ali Al-Sayed	Referred by Prof:	Khaled Al-Husseini
Age / Sex:	56 year / Female	Exam Date:	02 Apr 2026
Patient ID:	2140002820 Acc 1864270	Printed:	04 Apr 2026 09:42:45 AM

PT

V) **Thyroid gland:**

- Metabolically active bilateral heterogeneous thyroid lobes still achieving SUV max of 6 on the right side and SUV max of 5.6 on the left side, possibly thyroiditis for ultrasound and laboratory correlation.

VI) **The rest of the body:**

- Enlarged liver (17.5 cm) with no metabolically active FDG avid focal lesions. Minimal prominence of the biliary channels.
- Average sized spleen with no metabolically active focal lesions.
- No FDG avid sizable pulmonary masses or nodules. Bilateral basal pulmonary atelectatic bands.
- Regressive course of the diffuse increase in metabolic activity of the large bowel loops achieving max SUV~ 4.9 (compared to SUV max of 10.3) showing diverticular outpouchings.
- No metabolically active lytic/sclerotic osseous lesions.

Side Findings:

- Port a cath seen in place.

Still seen

- Cardiomegaly for echocardiography correlation.
- Elevated right copula of the diaphragm.
- Bilateral renal back pressure changes more on the right side.
- Spondylo-degenerative changes of the spine with mild anterolithesis of L4 over L5 for MRI correlation.

PET Scan
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Visit Number:	71026507395	Client Name:	
Patient Name:	Dalia Ali Al-Sayed	Referred by Prof:	Khaled Al-Husseini
Age / Sex:	56 year / Female	Exam Date:	02 Apr 2026
Patient ID:	2140002820 Acc 1864270	Printed:	04 Apr 2026 09:42:45 AM

PT

VII) **Normal physiological tracer bio-distribution is seen over cerebral hemispheres, heart, bowel, bone marrow and urinary tract.**

CONCLUSION:

Follow up PET/CT study revealed:

- **Mild metabolic regression with rather stationary size** duodenal circumferential thickening.
- **Regressive course** of the ill-defined retro-peritoneal mass (possibly amalgamated lymphadenopathy) and peritoneal thickening/nodularity.
- **Stable appearance of the right adnexal mass.**
- **Metabolic regression** of the rectal mural thickening.
- **No overt FDG avid breast masses**; however better evaluation by sono-mammography is recommended.

Dr.Ahmed Eid, FEBR, M.Sc

Prof. Ahmed Wafaie MD, FRCR

PET Scan
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Prof. MOHAMED TARIF HAMZA
Clinical Pathology/Hematopathology
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120 El-Thawra St. - Heliopolis - Cairo
Phone No. 26904454 - 26904459

MEDICAL LABORATORIES TS

معامل ا.د./ طريف حمزة سلام

دكتور
محمد طريف حمزه سلام
أستاذ التحاليل الطبيه و أمراض الدم
بكلية الطب و مستشفيات جامعة عين شمس
١٢٠ شارع الثورة - مصر الجديدة
ت: ٢٦٩٠٤٤٥٩ / ٢٦٩٠٤٤٥٤

Name MRS. DALYA ALY EL SAYED	Patient No	Ref. No	Date	Requested By:
	494402	494402	12/04/2026	PROF. KHALED EL HUSSAINY

EXAMINATION OF BONE MARROW ASPIRATE

The iliac marrow of this patient shows normal degree of cellularity. Granulocytopoiesis is depressed and shows ncreased early stages with mild dysplastic changes in the form of hyposegmentation and hypogranularity . Myeloblasts are 2% f the nucleated marrow elements.

Erythropoiesis is active with predominance of late stages with mild dysplasia. Iron stain shows increase in iron tores with no ringed sideroblasts.

The myeloid : erythroid ratio is increased (1: 2.4).

Lymphocytes are normal in count (10%) and normal in morphology. No lymphoma cells. No lymphoblasts.

Megakaryocytes are increased with many dysplastic forms. Free platelets are decreased in count.

Plasma cells are normal.

No haemoparasites.

No evidence of myelofibrosis.

No leukaemic, lymphomatous or nonhaematopoietic neoplastic infiltrations.

This marrow aspirate shows active haematopoiesis and absence of malignant infiltrations. The nemia in this case is mostly related to the patient's condition and/or therapeutic drugs with econdary dysplasia.

Trephine biopsy is separately reported.

Scanned with CamScanner
Bone Marrow Aspiration [[DOCTYPE:BONE_MARROW_ASP...

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MEDICAL LABORATORIES

معامل ا.د./ طريف حمزة سلام

دكتور
محمد طريف حمزه سلام
أستاذ التحاليل الطبيه و أمراض الدم
بكلية الطب و مستشفيات جامعة عين شمس
١٢٠ شارع الثورة - مصر الجديدة
ت: ٢٦٩٠٤٤٥٩ / ٢٦٩٠٤٤٥٤

Name MRS. DALYA ALY EL SAYED	Patient No 494402	Ref. No 494402	Date 12/04/2026	Requested By: PROF. KHALED EL HUSSAINY

BONE MARROW CORE BIOPSY EXAMINATION

Specimen: 2.5 cm core biopsy from right posterior iliac bone (ID: 26-816).

Comment: The bone marrow is normocellular for age (average cellularity ~ 50%).
The myeloid series shows normal maturation sequence.
The erythroid series is stimulated with normoblastic maturation with predominance of late forms.
Megakaryocytes are increased with many dysplastic forms.
No abnormal lymphoid or plasma cell clusters.
No Reed Sternberg cells.
Normal macrophages.
No evidence of granuloma, fibrosis or non-haematopoietic infiltration.

Conclusion: Normocellular bone marrow with trilineage haematopoiesis. No infiltrations were detected at the specimen examined.

One stained slide was given to the patient

Scanned with CamScanner
Bone Marrow Aspiration [[DOCTYPE:BONE_MARROW_ASP...

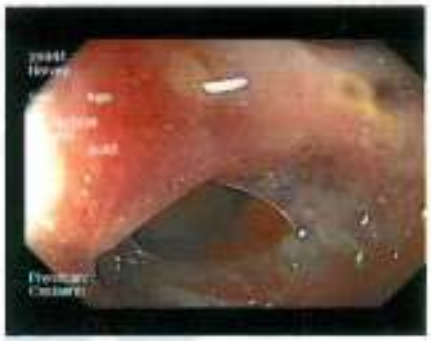
Colonoscopy Report

ENDOSCOPE

Patient Name:	داليا علي السيد	ID	26695
Endoscopist:	Dr Ramy Saeed	DOB	56 Y
2nd Operator		Age:	
Referred By:	-	Date	08/01/2026











INDICATION FOR EXAMINATION

PROCEDURE PERFORMED

Diagnostic

MEDICATIONS

FINDINGS

Colon:

SUBOPTIMAL COLONOSCOPE WAS DONE TILL TRANSVERSE COLON THE PATIENT NOT WELL PREPARED WITH MULTIPLE VARIABLE SIZED DIVERTICULAE THE SCOPE WAS NOT COMPLETED FOR FEAR OF PERFORATION

Conclusions

MULTIPLE VARIABLE SIZED COLONIC DIVERTICULAE
INTERNAL PILES

Recommendations

Endoscopist: Dr Ramy Saeed

ENDOSCOPE الاستقبال

Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]

Upper Endoscopy Report

Patient Name:	داليا علي السيد	ID	26695
Endoscopist:	Dr Ramy Saeed	DOB	56 Y
2nd Operator		Age:	
Referred By:	-	Date	08/01/2026

INDICATION FOR EXAMINATION

PROCEDURE PERFORMED

Diagnostic

MEDICATIONS

FINDINGS

Esophagus:

LOWER END MUCOSAL BREAKS LESS THAN 5MM
SMALL SLIDING HIATUS HERNIA

Stomach:

DIFFUSE HYPEREMIA OF THE WHOLE MUCOSA WITH DEFORMED PYLORIC RING

Pylorus:

DEFORMED

Duodenum:

CIRCUMFERENTIAL HYPEREMIC MUCOSA BIOPSIED AT JUNCTION BETWEEN BULB AND SECOND PART THE SCOPE PASSED HARDLY

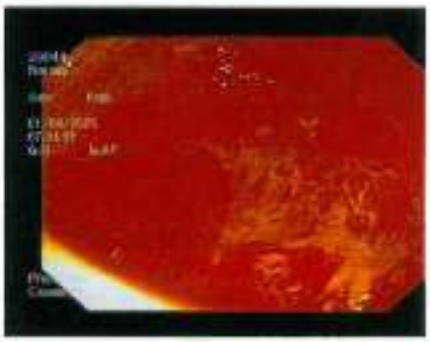
Conclusions

PARTIAL GASTRIC OUTLET OBSTRUCTION
SMALL SLIDING HIATUS HERNIA

Recommendations

Endoscopist: Dr Ramy Saeed











ENDOSCOPE
Bone Marrow Aspiration
[[DOCTYPE:BONE_MARROW_ASPIRATION]]





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ورم بالمعده
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National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]